MISH 38
Commercial Drivers Drug and Alcohol Program

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Drug & Alcohol Program

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MH CONSTRUCTION INDUSTRIAL SAFETY AND HEALTH PROGRAM

Title

COMMERCIAL DRIVERS DRUG AND ALCOHOL PROGRAM

Number

Revision 1 Approved By Jeremy Thamert 03/01/2025 Name Date Effective Date 3/1/2025 Page 1 of 19

Drivers are an extremely valuable resource for MH Construction‘s business. Their health and safety is a serious Company concern. Drug or alcohol use may pose a serious threat to driver health and safety. It is, therefore, the policy of the Company to prevent substance use or abuse from having an adverse effect on our drivers. The company maintains that the work environment is safer and more productive without the presence of alcohol, illegal or inappropriate drugs in the body or on company property. Furthermore, drivers have a right to work in an alcohol and drug-free environment and to work with drivers free from the effects of alcohol and drugs. Drivers who abuse alcohol or use drugs are a danger to themselves, their coworkers and the Company's assets. The adverse impact of substance abuse by drivers has been recognized by the federal government. The Federal Highway Administration (FHWA) has issued regulations, which require the company to implement a controlled substance testing program. The Company will comply with these regulations and is committed to maintaining a drug-free workplace. All drivers are

advised that remaining drug-free and medically qualified to drive are conditions of continued employment with the Company. Specifically, it is the policy of the Company that the use, sale, purchase, transfer, possession or presence in one's system of any controlled substance (except medically prescribed drugs) by any driver while on company premises, engaged in company business, while operating company equipment, or while under the authority of the Company is strictly prohibited.

FHWA states that mandatory testing must apply to every person who operates a commercial motor vehicle in interstate or intrastate commerce and is subject to the CDL licensing requirement.

The execution and enforcement of this policy will follow set procedures to screen body fluids (urinalysis), conduct breath testing, and/or search all driver applicants for alcohol and drug use, and those drivers suspected of violating this policy who are involved in a US Department of Transportation (DOT) reportable accident or who are periodically or randomly elected pursuant to these procedures. These procedures are designed not only to Detect violations of this policy, but to ensure fairness to each driver. Every effort will be made to maintain the dignity of drivers or driver applicants involved. Disciplinary action will, however, be taken as necessary.

Neither the policy nor any of its terms are intended to create a contract of employment or to contain the terms of any contract of employment. The Company retains the sole right to change, amend or modify any term or provision of this policy without notice. This policy is effective January 1, 2019, and will supersede all prior policies and statements relating to alcohol or drugs.

1.0 PURPOSE The purpose of this administrative guide is to set forth the procedures for the implementation of Controlled substances and alcohol use and testing of driver applicants and current drivers pursuant to The Alcohol and Drug Abuse Policy. These procedures are intended as a guide only, and are in no way intended to alter any existing relationship between MH Construction and any driver.

MH Construction Group alcohol and drug program administrator designated to monitor, facilitate, and answer questions pertaining to these procedures is Alex Linde or Matt Youngberg.

2.0 DEFINITIONS When interpreting or implementing these procedures, or the procedures required by he Federal Highway Administration (FHWA) controlled substance testing regulations the following definitions apply: "Alcohol" means the intoxicating agent in beverage alcohol, ethyl alcohol, or other low molecular weight alcohols including methyl and isopropyl alcohol. "Alcohol concentration (or content)" means the alcohol in a volume of breath expressed in terms of grams of alcohol per 210 liters of breath as indicated by an evidential breath test under this part.

"Collection site" means a place where individuals present themselves for the purpose of providing breath, body fluid, or tissue samples to be analyzed for specified controlled substances. This site must possess all necessary personnel, materials, equipment, facilities and supervision to provide for the collection, security, temporary storage and transportation or shipment of the samples to a laboratory.

"Commercial motor vehicle" means a motor vehicle or combination of motor vehicles used in commerce to transport passengers or property if the motor vehicle:

1 Has a gross combination weight rating of 26,001 or more pounds

2 Has a gross vehicle weight rating of 26,001 or more pounds

3 Is designed to transport 16 or more passengers, including the driver

4 Is of any size and is used in the transportation of materials found to be hazardous for the purposes of the Hazardous Materials Transportation Act and which require the motor vehicle to be placarded under the Hazardous Materials Regulations (49 CFR, Part 172, 1308).

"Driver" means any person who operates a commercial motor vehicle. This includes, but is not limited to: Full time, regularly employed drivers; casual, intermittent or occasional drivers; leased drivers and independent, owner-operator contractors who are either directly employed by or under lease to an employer or who operate a commercial motor vehicle at the direction of or with the consent of an employer. For the purpose of pre-employment pre-duty testing only, the term "driver" includes a person applying to an employer to drive a commercial motor vehicle.

"Drug" means any substance (other than alcohol) that is a controlled substance as defined in the section and 49 CFR, Part 40.

"FHWA" means the Federal Highway Administration; US Department of Transportation.

"Owner-operator(s)" means a driver(s) who has been contracted for services with the Company. For the purposes of these procedures and the Company's Alcohol and Drug Abuse Policy, owner-operators are not to be considered employees, but will be required to participate in the Company's Alcohol and Drug Abuse Policy like all Company employee drivers.

"Medical review officer" (MRO) means a licensed MD or DO with knowledge of drug abuse disorders that is employed or used by a motor carrier to conduct drug testing in accordance with this part.

"Performing a safety-sensitive function" means a driver is considered to be performing a safety-sensitive function during any period in which he/she is actually performing, ready to perform, or immediately available to perform any safety-sensitive functions.

"Random selection process" means that alcohol and drug tests are unannounced; that every driver of a motor carrier subject to test-tests conducted annually shall equal or exceed twenty-five percent (25%) for alcohol tests and fifty percent (505) for drug test of the total number of drivers subject to testing of a motor carrier.

"Reasonable cause" means that the motor carrier believes the actions or appearance or conduct of a commercial motor vehicle driver who is on duty as defined below, are indicative of the use of a controlled substance.

"Safety-sensitive function" means any of those on-duty functions set forth in CFR 49, Section 395.2.

"On duty time" means all time from the time a driver begins to work or is required to be in readiness to work until the time he/she is relieved from work and all responsibility for performing work. "On duty time" shall include:

All time at a carrier or shipper plant, terminal, or facility, or other property, or on any public Property waiting to be dispatched, unless the driver has been relieved from duty by the motor carrier;

• All time inspecting, servicing, or conditioning any commercial motor vehicle at any time;

• All driving time;

• All time, other than driving time, in or upon any commercial motor vehicle except time spent resting in a sleeper berth;

• All time loading or unloading a vehicle, supervising, or assisting in the loading or unloading, attending a vehicle being loaded or unloaded, remaining in readiness to operate the vehicle, or in giving or receiving receipts for shipments loaded or unloaded

• All time repairing, obtaining assistance, or remaining in attendance upon a disabled vehicle

3.0 SUBSTANCES PROHIBITED/PRESCRIPTION MEDICATIONS

3.1 Alcohol use means the consumption of any beverage, mixture, or preparation, including any medication containing alcohol which, when consumed, causes an alcohol concentration in excess of those prescribed by Part 382, Subpart B, (FMCSR) and Section IV of this policy.

3.2 Controlled substances: In accordance with FHWA rules, urinalysis will be conducted to detect the presence of the following substances:

• Marijuana

• Cocaine

• Opiates

• Amphetamines

• Phencyclidine (PCP)

Detection levels requiring a determination of a positive result shall be in accordance with the guidelines adopted by the FHWA in accordance with the recommendations established by the 49 CFR, Part 40.

3.3 Prescription medications: Drivers taking legally prescribed medications issued by a licensed health care professional familiar with the driver's work-related responsibilities must report such use to their immediate supervisor or dispatcher, and may be required to present written evidence from the health care professional which describes the effects such medications may have on the driver's ability to perform his/her tasks. In the sole discretion of the alcohol and drug program administrator, a driver may be temporarily removed, with pay, from a safety-sensitive position if deemed appropriate.

4.0 PROHIBITIONS

4.1 Alcohol Prohibitions The new alcohol rule prohibits any alcohol misuse that could affect performance of a safety- Sensitive function, including:

• Use while performing safety-sensitive functions.

• Use during the 4 hours before performing safety-sensitive functions.

• Reporting for duty or remaining on duty to perform safety-sensitive functions with an alcohol concentration of 0.04 or greater.

• Possession of alcohol, unless the alcohol is manifested and transported as part of a shipment. This includes the possession of medicines containing alcohol (prescription over-the-counter), unless the packaging seal is unbroken.

• Use during 8 hours following an accident, or until he/she undergoes a post accident drug test

• Refusal to take a required test.

NOTE: A driver found to have an alcohol concentration of 0.02 or greater but less than 0.04 shall not be permitted to perform, safety-sensitive functions for at least 24 hours. The other consequences imposed by the regulations and discussed below do not apply. However, documentation of this test constitutes written warning that company policy has been violated, and the next occurrence could result in disqualification of a driver.

4.2 Drug Prohibitions The regulations prohibit any drug use that could affect performance of safety-sensitive functions, including:

• Use of any drug, except by doctor's prescription, and the only if the doctor has advised the driver that the drug will not adversely affect the driver's ability to safely operate the CMV:

• Testing positive for drugs; and

• Refusing to take a required test.

• All drivers will inform the alcohol and drug program administrator of any therapeutic drug use prior performing a safety-sensitive function.

5.0 DRIVER APPLICANT AND CURRENT DRIVER TESTING

5.1 Applicant Testing:

5.1.1 All driver applicants will be required to submit to and pass a urine drug test as a condition of employment.

5.1.2 Job applicants who are denied employment because of a positive test may reapply for employment after 1 year.

5.1.3 Offers of employment are made contingent upon passing the Company's medical review, including the alcohol and drug test. Driver applicants who have received firm employment offers are to be cautioned against giving notice at their current place of employment, or incurring any costs associated with accepting employment with the Company until after medical clearance has been received. All newly hired drivers shall be on probationary status for 30 days, continent upon medical clearance for illicit controlled substance or alcohol usage. Under no circumstances may a driver perform a safety-sensitive function until a confirmed negative result is received.

5.1.4 Driver applicant drug testing shall follow the collection, chain-of-custody and reporting procedures as set forth in 49 CFR, Part 40.

5.2 Owner-Operators:

5.2.1 Owner-operators engaged by the Company are not employees of the Company, nor are they to be considered as such under this Policy. However, every owner- operator engaged to provide services to the Company must agree to, and successfully participate in the Company's alcohol and drug testing program. All owner-operator agreements will be entered into by he the Company

contingent upon the operators' successful completion of urinalysis and breath analysis under all phases of the Company's program, and are contingent upon the owner-operator's continued status as a medically qualified driver.

5.2.2 The term "driver" as used in these procedures includes owner-operators.

5.3 Employee Drivers: Under all circumstances, when a driver is directed to provide either a breath test or urine sample (Appendix C) in accordance with these procedures, he/she must immediately comply as instructed. Refusal will constitute a positive result, and the driver will be immediately removed from the safety-sensitive function, and will be subject to further discipline or termination as appropriate.

5.3.1 Suspicion-based Testing:

5.3.1.1 Reasonable Suspicion: If a driver is having work performance problems or displaying behavior that may be alcohol or drug-related, or is otherwise demonstrating conduct that may be in violation of the Policy where immediate management action is necessary, a supervisor or dispatcher, with the concurrence of the alcohol and drug program administrator, will Require that driver is to submit to a breath test or urinalysis. The following conditions are signs of possible alcohol or drug use (not all-inclusive):

• Abnormally dilated or constricted pupils

• Glazed stare -redness of eyes

• Flushed Face

• Change of speech (i.e. faster or slower)

• Constant sniffing

• Increased absences

• Redness under nose sudden weight loss

• Needle marks

• Change in personality (i.e. paranoia)

• Increased appetite for sweets

• Forgetfulness -performance faltering -poor concentration

• Borrowing money from coworkers or seeking an advance of pay or other unusual display of need for money

• Constant fatigue or hyperactivity

• Smell of alcohol slurred speech

• Difficulty walking

• Excessive, unexplained absences

• Dulled mental processes

• Slowed reaction rate

5.3.2 Supervisor Required Action Supervisors or dispatchers must take action if they have reason to believe one or more of the above-listed conditions is indicated, and that the substance abuse is affecting a driver's job performance or behavior in any manner. A supervisor or dispatcher observing such conditions will take the following actions immediately:

• Confront the employee involved, and keep under direct observation until the situation is resolved.

• Secure the alcohol and drug program administrator's concurrence to observations; job performance and company policy violations must be specific.

• After discussing the circumstances with the supervisor or dispatcher, the alcohol and drug program administrator will arrange to observe or talk with the driver. If he/she believes, after observing or talking to the driver, that the conduct or performance problem could be due to substance abuse, the driver will be immediately informed that continued refusal will result in disqualification from performing any safety-sensitive function.

• Employee will be asked to release any evidence relating to the observation for further testing. Failure to comply may subject the employee to subsequent discipline or suspension from driving duties. All confiscated evidence will be receipted for with signatures or both the receiving supervisor, as well as the provider.

• If upon confrontation by the supervisor or dispatcher, the driver admits to use but requests assistance, the alcohol and drug program administrator will arrange for assessment by an appropriate substance abuse professional (SAP). Reassignment to the driver position is conditional to completing the SAP's guidelines and return-to-work testing.

5.3.2.1 The supervisor or dispatcher shall, within 24 hours or before the results of the controlled substance test are released, document the particular facts related to the behavior or performance problems, and present such documentation to the alcohol and drug program administrator.

5.3.2.2 The drug and alcohol program administrator will remove or cause the removal of the driver from the Company-owned vehicle and ensure that the driver is transported to an appropriate collection site an thereafter to the driver's residence or, where appropriate, to a place of lodging. Under no circumstances will that driver be allowed to continue to drive a Company vehicle or his/her own vehicle until a confirmed negative test result is received.

5.3.2.3 If, during the course of employment, the driver acknowledges substance abuse problem and requests assistance, the problem may be treated as if it were an illness, subject to the provisions set forth below:

• The decision to seek diagnosis and accept treatment for the substance abuse problem is the responsibility of the driver:

• The diagnosis and prescribed treatment of the driver's condition will be determined by health care professionals designated by the alcohol and drug program administrator in conjunction with the driver's physician; and

• The driver might be placed on medical leave for a predetermined period recommended by those medical professionals if the SAP determines that such action is appropriate.

NOTE: The employee is responsible for all medical costs associated with treatment.

5.3.3 Post Accident Drug Testing

Currently, federal regulations place the burden of compliance with post- accident alcohol and drug testing regulations on the driver. Therefore, all drivers are required to provide a breath test and a urine specimen to be tested for the use of controlled substances "as soon as practicable" after an accident. The driver shall remain readily available for such testing or may be deemed by the alcohol and drug program administrator to have refused to submit to testing. No alcohol may be consumed for 8 hours after the accident or until a test is conducted. If the driver is seriously injured and cannot provide a specimen at the time of the accident, he/she shall provide the necessary authorization for obtaining hospital reports and other documents that would indicate whether there were any controlled substances in his/her system.

5.3.3.1 An accident is defined by FHWA regulations as an accident, which results in the death of a human being or bodily injury to a person who, as a result of the injury, immediately receives medical treatment away from the scene of the accident; or which has had one of the vehicles towed from the Scene of the accident. Except for a fatality accident, verification of the driver's responsibility in the above accident must be established by a citation to the driver.

5.3.3.2 Adherence by drivers to post-accident specimen collection requirements is a condition of continued employment. The failure of an owner-operator to comply with DOT post-accident and specimen collection rules will be considered a breach of his/her contract with the Company, and the contract is invalid until appropriate substance abuse professional counseling has been completed.

5.3.4 Random Testing: The Company will conduct random testing for all covered drivers as follows:

MH Construction Drivers are listed in a pool managed and operated by Lourds Occupational Health Foundation. Lords will randomly select employees from their computer system and will notify the drug test administrator. The random testing, will provide for alcohol testing of at least 25% and for drug testing of at least 50% of all covered drivers.

5.3.4.1 Once notified, a driver must proceed immediately to the assigned collection site.

5.3.4.2 The alcohol and drug program administrator will be responsible for designating the appropriate Substance abuse professional who, in conjunction with the driver's physician, will diagnose the problem and recommend treatment.

5.3.4.3 The driver's successful completion of the approved treatment program is a condition of continued employment as a driver.

5.3.4.4 Following successful completion of any approved treatment program, the driver will be required to submit to at least six random drug tests during the first year, and follow-up testing may be conducted for up to 60 months. Failure to adhere to this condition is grounds for immediate termination.

5.3.4.5 All supervisors or dispatchers will receive training to assist them in identifying alcohol and drug se behavioral characteristics.

NOTE: The employee is responsible for all medical costs associated with treatment.

5.3.5 Return-to-Duty Testing: Before a driver returns to duty requiring the performance of a safety-sensitive function after engaging in conduct prohibited by this policy and Part 382, Subpart B (FMCSA), the driver shall undergo a return to duty alcohol test with a result of less than a 0.02 BAC and receive a confirmed negative result from a controlled substance urinalysis test.

5.4 COLLECTION OF BREATH AND URINE SPECIMENS AND LABORATORY ANALYSIS

5.4.1 Qualified Breath Alcohol Technician according to 49 CFR, Part 40 procedures, will conduct either on site or at a prearranged location

5.4.1.1 Breath alcohol testing: Refusal to complete and sign the testing form or refusal to provide breath will be considered a positive test, and the driver will be removed from a safety-sensitive function until resolved.

5.4.1.2 Specimen Collection: Specimen collection will be conducted in accordance with applicable state and federal law. The collection procedures will be designed to ensure the security and integrity of the specimen provided by each driver, and those procedures will strictly follow federal chain-of-custody guidelines. Moreover, every reasonable effort will be made to maintain the dignity of each driver submitting a specimen for analysis in accordance with these procedures.

5.4.1.3 Laboratory Analysis: As required by FHWA regulations, only a laboratory certified by Department of Health and Human Services (DHHS) to perform urinalysis for the detection of controlled substance.

The Company will retain the presence of controlled substances. The laboratory will be required to maintain strict compliance with federally approved chain-of-custody procedures, quality control, maintenance and scientific analytical methodologies.

5.5 CONSEQUENCES: APPEAL OF TEST RESULTS

5.5.1 Alcohol and drug abuse may not only threaten the safety and productivity of all employees at MH Construction but causes serious individual health consequences to those who use them. Attachment #1 outlines several personal consequences, which may result after abuse of controlled substances. Any confirmed actions prohibited by this program while performing a safety-sensitive function or refusing to take a breath/urine test, will be grounds for disqualification as a driver.

5.5.2 A driver testing positive for alcohol or drug use is subject to disqualification. Refusal to submit to testing will also be considered a positive.

5.5.3 Refusal may be defined as not providing a breath sample or urine as directed, neglecting to sign appropriate control forms, using alcohol within 8 hours of an accident, or engaging in conduct that clearly obstructs the testing process.

5.5.4 Any driver testing positive for the presence of a controlled substance will be contacted by the Company's MRO. The driver will be allowed to explain and present medical documentation to explain any permissible use of a drug. All such discussions between the driver and the MRO will be confidential. The Company will not be a party to, or have access to, matters discussed between the driver and the MRO. If medically supportable reasons exist to explain the positive result, the MRO will report the test result to the Company as a negative.

5.5.5 Within 72 hours after the driver has been notified of a positive test result for drugs, he/she may request a retest of the split sample. This signed request will be provided to the MRO in writing, which will then initiate the new laboratory analysis. If the subsequent laboratory detects a different result, the MRO, will void the test and drug program administrator will be notified. A retest may be initiated as appropriate.

5.6 CONFIDENTIALITY

5.6.1 Under no circumstances, unless required or authorized by law, will alcohol or drug testing information or results for any employee or applicant be released without written request from the applicable employee.

5.6.2 Drivers are entitled, upon written request, to obtain copies of any records pertaining to the driver's use of alcohol or controlled substances, including any records pertaining to his or her alcohol or controlled substance test.

5.6.3 Collection of breath and urine samples must always be documented and sealed with a tamper-proof sealing system in the presence of the driver, to ensure that all tests can be correctly traced to the driver.

5.6.4 Drug test analysis from the approved laboratory will be forwarded directly to the Medical Review Officer assigned by the alcohol and drug program administrator.

5.6.5 Alcohol test results will be forwarded by the MRO to the alcohol and drug program administrator for confidential record keeping.

APPENDIX A ALCOHOL AND DRUG EFFECTS

Section 382.601 (b)(11) FMCSA mandates that all employees be provided with training material discussing the effects of alcohol and controlled substance use on an individual's health, work, and personal life.

This attachment is intended to help individuals understand the personal consequences of substance abuse.

ALCOHOL

Although used routinely as beverage for enjoyment, alcohol can also have negative physical and mood-altering effects when abused. These physical or mental alterations in a driver may have serious personal and public safety risks.

Health Effects An average of three or more servings per day of beer (12 ounces), whiskey (1 ounce), or wine (6 ounces) Overtime, may result in the following health hazards:

• Dependency

• Fatal liver diseases

• Kidney failure.

• Pancreatitis Ulcers

• Decreased sexual function

• Increased cancers of the mouth, pharynx, esophagus, rectum, breast, and malignant melanoma

• Spontaneous abortion and neonatal mortality

• Birth defects

Social Issues

• People who drink prior to the crime commit 2/3 of all homicides.

• 2- 3% of the driving population are legally drunk at anyone time. This rate doubles at night and on weekends.

• 2/3 of all Americans will be involved in an alcohol-related vehicle accident during their lifetime.

• The separation and divorce rate in families with alcohol dependency problems is 7 times the average.

• 40% of family court cases are alcohol-related.

• Alcoholics are 15 times more likely to commit suicide.

• More that 60% of burns, 40% of falls, 69% of boating accidents, and 76% of private aircraft accidents are alcohol accident-related.

• Over 17, 000 fatalities occurred in 1993 in highway accidents, which were alcohol-related. This was 43% of all highway fatalities.

• 30,000 people will die each year from alcohol-caused liver disease.

• 10,000 people will die each year due to alcohol-related brain disease or suicide.

• Up to 125,000 people die each year due to alcohol-related conditions or accidents.

Workplace Issues

• It takes one hour for the average person (150 pounds) to process one serving of alcohol from the body.

• Impairment can be measured with as little as two drinks in the body.

• A person who is legally intoxicated is 6 times more likely to have an accident than a sober person.

ALCOHOL'S TRIP THROUGH THE BODY

Mouth and Esophagus: Alcohol is an irritant to the delicate linings of the throat and food pipe. It burns as it goes down.

Stomach and Intestines: Alcohol has an irritating effect on the stomach’s protective lining, resulting in gastric or duodenal ulcers. This condition, if it becomes acute, can cause peritonitis, or perforation of the stomach wall. In the small intestine, alcohol blocks absorption of such substances as thiamine, folic acid, fat, vitamin 81, vitamin 812, and amino acids. Bloodstream: 95% of the alcohol taken into the body is absorbed into the bloodstream through the lining of the stomach and duodenum. Once in the bloodstream, alcohol quickly goes to every cell and tissue in the body. Alcohol causes red blood cells to clump together in sticky wads, slowing circulation and depriving tissues of oxygen. It also causes anemia by reduction red blood cell production. Alcohol slows the ability of white cells to engulf and destroy bacteria and degenerates the clotting ability

of blood platelets. Pancreas: Alcohol irritates the cells of the pancreas, causing them to swell, thus blocking the flow of digestive enzymes. The chemicals, unable to enter the small intestine, begin to digest the pancreas, leading to acute hemorrhagic pancreatitis. One out of five patients who develop this disease die during the first attack. Pancreatitis can destroy the pancreas and cause a lack of insulin, thus resulting in diabetes. Liver: Alcohol inflames the cells of the liver, causing them to swell and block the tiny canal to the small intestines. This prevents bile from being filtered properly through the liver. Jaundice develops, turning the whites of the eyes and skin yellow. Each drink of alcohol increases the number of live cells destroyed, eventually causing cirrhosis of the liver. This disease is eight times more frequent among alcoholics than among non-alcoholics. Heart: Alcohol causes inflammation of the heart muscle. It has a toxic effect on the heart and causes increased amounts of fat to collect, thus disrupting its normal metabolism. Urinary Bladder and Kidneys: Alcohol inflames the lining of the urinary bladder making it unable to stretch properly. In the kidneys, alcohol causes increased loss of fluids through its irritating effect. Sex Gland: Swelling of the prostate gland caused by alcohol interferes with the ability of the male to perform sexually. It also interferes with the ability to climax during intercourse. Brain: The most dramatic and noticed effect of alcohol is on the brain. It depresses brain centers, producing progressive in coordination: confusion, disorientation, stupor, anesthesia, coma, and death. Alcohol kills brain cells and brain damage is permanent Drinking over a period of time causes loss of memory, judgment and learning ability.

DRUGS

Marijuana

Health Effects

• Emphysema-Like conditions.

• One joint of marijuana contains cancer-causing substances equal to 1/2 pack of cigarettes

• One joint causes the heart to race and be overworked. People with heart conditions are at risk.

• Marijuana is commonly contaminated with the fungus Aspergillums that can cause serious respiratory tract and sinus infections.

• Marijuana lowers the body's immune system response, making users more susceptible to infection.

• Chronic smoking causes changes in brain cells and brain waves

• The brain does not work as efficiently or effectively. Long-term brain damage may occur.

• Tetrahydrocannabinol (THC) and 60 other chemicals in marijuana concentrate in the ovaries and testes.

• Chronic smoking of marijuana in males causes a decrease in testosterone and an increase in estrogen, the female hormone. As a result, the sperm count is reduced, leading to temporary sterility.

• Chronic smoking of marijuana in females causes a decrease in fertility.

• A higher than normal incidence of stillborn babies, early termination of pregnancy, and higher infant mortality rate during the first few days of life are common in pregnant marijuana smokers.

• THC causes birth defects including brain damage, spinal cord, forelimbs, liver and water on the brain

Workplace Issues

• THC is stored in body fat and slowly released.

• Marijuana smoking has long-term effects on performance.

• Increased THC potency in modern marijuana dramatically compounds the side effects.

• Combining alcohol or other depressant drugs with marijuana increases the impairing effects of both.

Cocaine Used medically as a local anesthetic. When abused, it becomes a powerful physical and mental stimulant. The entire nervous system is energized. Muscles tense, heartbeats faster and stronger, and the body burns more energy. The brain experiences exhilaration caused by a large release of neurohormones associated with mood elevation.

Health Effects

• Regular use may upset the chemical balance of the brain. As a result, it may speed up the aging process causing damage to critical nerve cells. Parkinson's disease could also occur.

• Cocaine causes the heart to beat faster, harder, and rapidly increases blood pressure. It also causes spasms of blood vessels in the brain and heart. Both lead to ruptured vessels causing strokes and heart attacks.

• Strong dependence can occur with one "hit" of cocaine. Usually mental dependency occurs within days for "crack" or within several months for snorting coke. Cocaine causes the strongest mental' dependency of all the drugs,

• Treatment success rates are lower than with any other chemical dependency.

• Extremely dangerous when taken with other depressant drugs. Death due of overdose is rapid. Fatal effects are usually not reversible by medical intervention.

Workplace Issues

• Extreme mood and energy swings create instability. Sudden noise causes a violent reaction.

• Lapses in attention and ignoring warning signals increases probability of accidents.

• High cost frequently leads to theft and/or dealing.

• Paranoia and withdrawal may create unpredictable or violent behavior.

• Performance is characterized by forgetfulness, absenteeism, tardiness, and missing assignments.

Opiates Narcotic drugs, which alleviate pain and depress body functions and reactions.

Health Effects

• IV needle users have a high risk of contracting hepatitis or AID when sharing needle.

• Increased pain tolerance. As a result, a person may more severely injure him or herself and fail to seek medical attention as needed.

• Narcotic effects are multiplied when combined with other depressants causing an increased risk for Overdose.

• Because of tolerance, there is an ever-increasing need for more.

• Strong mental and physical dependency occurs.

• With increased tolerance and dependency combined, there is a serious financial burden for the users.

Amphetamines Amphetamines is a central nervous system stimulant that speeds up the mind and body.

Health Effects

• Regular use causes strong psychological dependency and increased tolerance.

• High doses may cause toxic psychosis resembling schizophrenia.

• Intoxication may induce a heart attach or stroke due to increased blood pressure.

• Chronic use may cause heart or brain damage due to severe constriction of capillary blood vessels.

• Euphoric stimulation increases impulsive and risk taking behavior, including bizarre and violent acts.

• Withdrawal may result in severe physical and mental depression.

Workplace Issues

• Since the drug alleviates the sensation of fatigue, it may be abused to increase alertness during periods of overtime or failure to get rest.

• With heavy use or increasing fatigue, the short-term mental or physical enhancement reverses and becomes impairment.

Phencyclidine (PCP)

• Often used as a large animal tranquilizer. Abused primarily for its mood altering effects. Low doses produce sedation and euphoric mood changes. Mood can rapidly change from sedation to excitation and a blank stare. Sudden noises or physical shocks may cause a "freak out" in which the person has abnormal strength, violent behavior, and an inability to speak or comprehend.

Health Effects

• The potential for accidents and overdose emergencies is high due to the extreme mental effects combined with the anesthetic effect on the body.

• PCP, when combined with other depressants, including alcohol, increases the possibility of an overdose.

• If misdiagnosed as LSD induced, and treated with Theorizing, can be fatal.

• Irreversible memory loss, personality changes, and thought disorders may result.

Workplace Issues Not common in workplace primarily because of the severe disorientation that occurs.

There are four phases to PCP abuse:

• Acute toxicity causing combativeness, catatonia, convulsions, and coma. Distortions of size, shape, and distorted perception are common.

• Toxic psychosis with visual and auditory delusions, paranoia and agitation.

• Drug induced schizophrenia.

• Induced depression, which may create suicidal tendencies and mental dysfunction.

APPENDIX B DRIVER NOTIFICATION LETTER

I certify that I have read and received a copy of the MH Construction policy on alcohol and Drug Testing procedures. I understand that as a condition of employment as a driver I must comply with these guidelines, and do agree that I will remain medically qualified by following these procedures. If I develop a problem with alcohol or drug abuse during my employment with MH Construction I will seek assistance through the current alcohol and drug Testing program administrator (the employee is responsible for all costs associated with treatment).

Name and Signature Date

Part 382 -Controlled Substances and Alcohol Use Testing applies to drivers of this company.

Company Name: MH Construction

Driver/Applicant Name:

_____ (Print) (First, M.I., Last)

Motor Carrier Safety Regulations. You are hereby notified the following test will be administered in compliance with the Federal Motor Carrier Regulations.

1.The test is scheduled:

Date:_____
Location:_____
Time:_____ _____

2. Check type of test: Alcohol and/or Controlled Substance 3. Check reason for test: Pre-employment Random Reasonable suspicion/cause Post-accident Return to Duty Follow-up

4. Appointment instructions/comments:

I understand as a condition of my employment with this company, the above test is required.

_____ _____ Driver/Applicant's Signature Date

_____ Witness By: Company Representative Date

Return this form to Jeremy Thamert or Steve McClary for Confidential filing.

MISH 38 — Commercial Drivers Drug and Alcohol Program

Scan to view this section online, download the PDF, or access the full MH Construction Safety Manual.

https://www.mhc-gc.com/resources/safety-manual/section/commercial-drivers-drug-and-alcohol-program